

AMBIENT AI REPORT:

ADOPTION, SCALE, AND IMPACT

Data and Insights
From 150+ Enterprise
Deployments

ABRIDGE

Generative AI is changing healthcare at an accelerating pace. Nearly all healthcare executives (95%) recently surveyed by Bain & Company, Bessemer Venture Partners, and Amazon Web Services now agree that “generative AI will transform the industry.”

Yet, according to the same survey, only 45% of AI applications have moved beyond the ideation stage and only 30% have made it into production. At provider organizations, only 30% are deploying AI across the enterprise, with most still in the pilot phase.

➤ Abridge partners have had significant success scaling ambient AI, and scale drives impact. Abridge is currently used by tens of thousands of clinicians at more than 150 of the largest and most complex health systems to generate millions of notes per month—while improving clinician and patient experiences and transforming revenue cycle management.

When so many promising AI concepts are still in the testing phase, how does Abridge scale across care settings, specialties, languages, and workflows to meet the needs of a diverse set of healthcare institutions across the U.S.?

In this report, we will share the steep adoption curves for five of our health system partners and show how these implementations deliver immediate impact to clinician wellbeing, patient experience, and financial ROI across many more.



SPEED TO SCALE:

Rapid Adoption at Five Major Health Systems

Abridge is currently operating a high volume of monthly implementations, with time to implementation typically just a few weeks. The fastest deployment occurred at Duke Health, one of the world's leading academic health centers headquartered in Durham, NC—completed in just five days¹. Duke's rapid adoption was driven by a focused effort to reduce documentation workload for clinicians and enhance the patient-clinician encounter experience.

Once implemented, Abridge partners often want to scale faster than initially planned because of immediate impact on clinician wellbeing and job satisfaction.

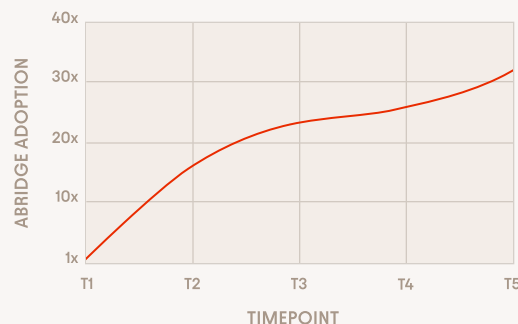
To meet this need for thousands of clinicians, Abridge has developed a unique implementation practice that enables rapid scale. Here are the current adoption curves of five Abridge partners².

Combined, these five implementations represent **thousands of clinicians** commencing use of Abridge over just a few months:

How to Read These Charts

Each Abridge implementation begins with a small cohort of early users (represented by "1x" on each chart). After experiencing success, partners decide to scale users at a pace of their choosing, represented in these charts as a multiple of the initial cohort.

FIG. 1
Duke Health



30x

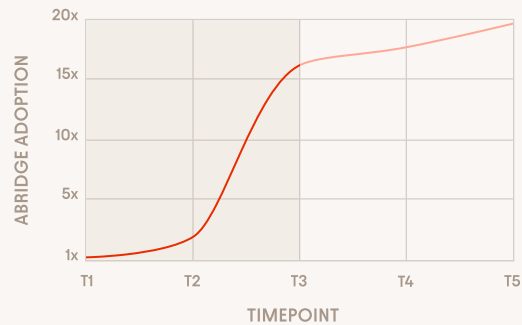
Launch to 30x
in four months

1. Average deployment time for SaaS technologies in healthcare ranges from 3-6 months.

2. Partner data and agreement terms are proprietary. Because of this, we are not sharing the names and actual numbers for all five implementations. These five implementations are representative of many Abridge has conducted. For details behind these numbers, please [contact Abridge](#).

FIG. 2

Corewell Health



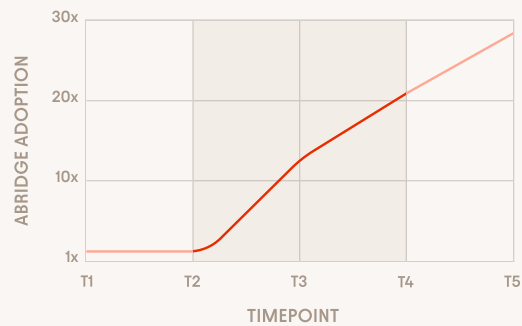
15x

Launch to 15x
in two months



FIG. 3

UNC Health



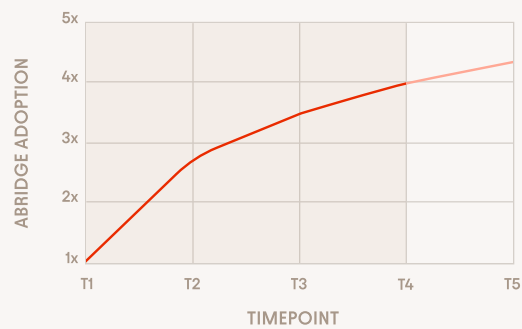
20x

Launch to 20x
in two months



FIG. 4

An internationally
recognized health
center of innovation



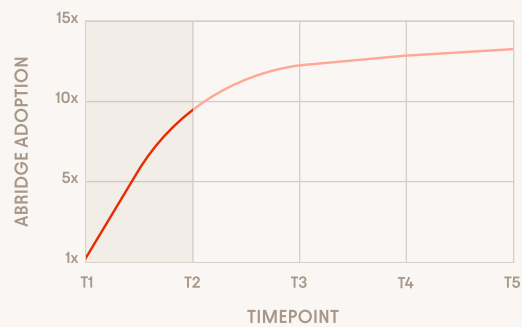
4x

Launch to 4x
in three months



FIG. 5

Johns Hopkins
Medicine



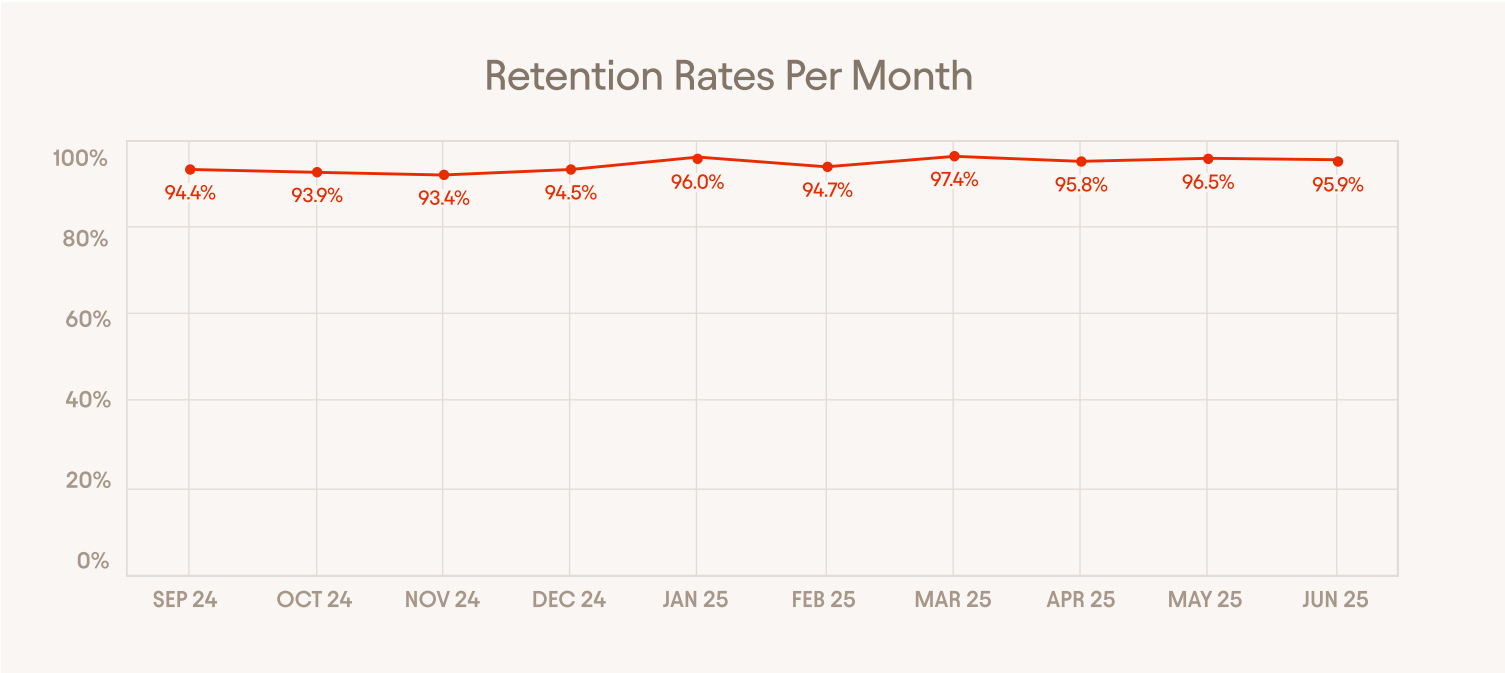
10x

Launch to 10x
in one month



Notice that each of these curves is different. Abridge scales at the speed of each organization, depending on its unique needs.

These rapid adoptions are supported by a product that clinicians love using: **Abridge month-over-month net retention rates, which include re-engaged clinicians, are consistently at levels of 95%+.**



"We're seeing an amazing response from our clinicians to Abridge. **The learning curve is fast**—just hit start, hit stop, and watch the data flow seamlessly into place."

Dr. William Carracino
Chief Medical Informatics Officer, Lee Health

SPEED TO IMPACT

Immediate Results From Ambient AI Implementation

Abridge measures impact across health systems in three primary categories:

01 Clinician Experience



02 Patient Experience



03 Revenue Cycle Management

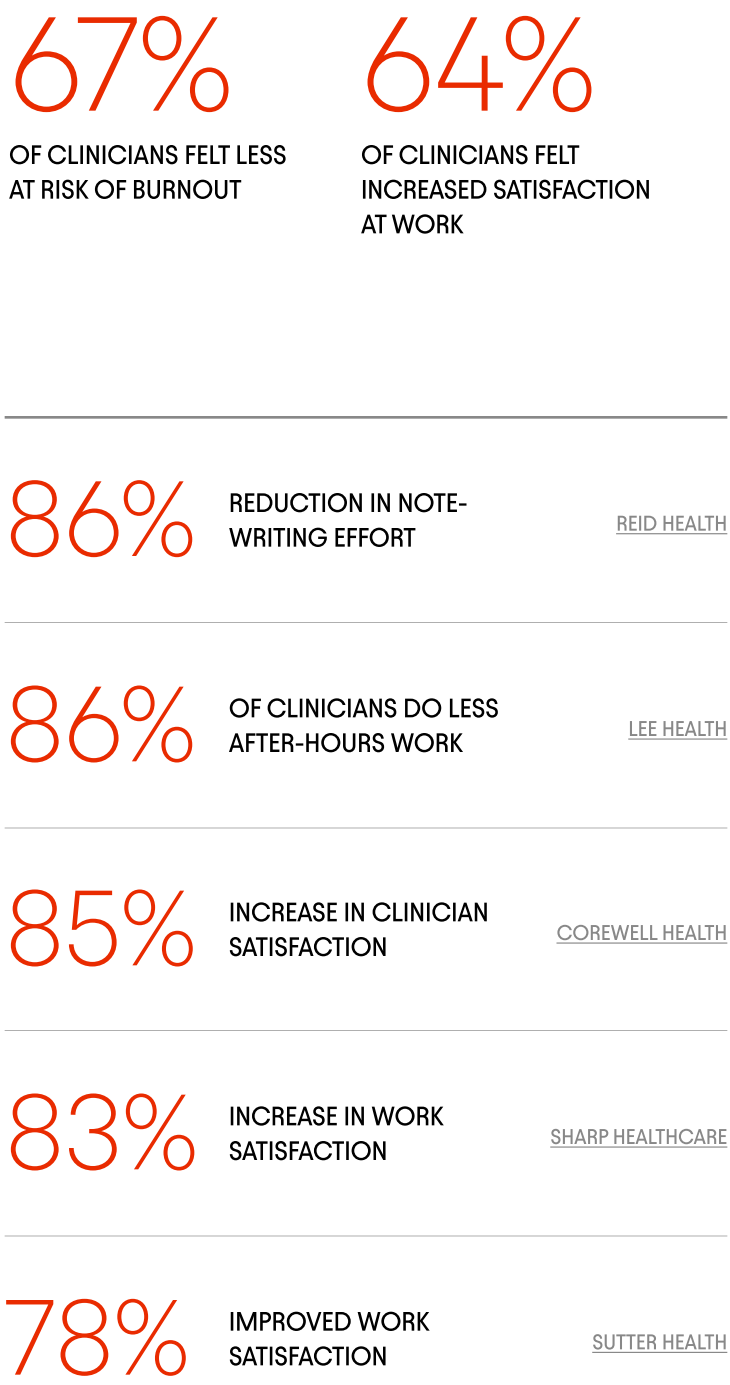


Clinician Experience

Nearly half of clinicians are experiencing burnout, according to the American Medical Association—and documentation burden is the leading cause. Abridge has been proven in peer-reviewed research to reduce burnout:



Using scientifically validated survey methodology, Abridge partner systems measure and share their own rapid success stories:



Clinician Experience



"I have never seen such an **immediate impact** from a technology solution."

Dr. Tanya Dansky
Senior Medical Director, MemorialCare Medical Group



"The reduction in cognitive load with Abridge has been incredibly freeing and is **giving me back precious time with my patients and family.**"

Dr. Kristin Jacob
Medical Director, Physician and APP Well-Being, Corewell Health



"The impact of Abridge on clinicians has been even better than I expected. A busy primary care doctor reported that this is the first time in 30 years she is finishing her notes by the end of clinic. **A surgeon told me that he used to spend 4-5 hours at the end of every day to finish documentation—now he's done in an hour.**"

Dr. Bonnie Boles
SVP, Chief Medical Information Officer, Tanner Health



Patient Experience



Clinicians who are more attentive and focused deliver better patient experiences, according to [research in the Journal of Family Medicine](#). By removing the cognitive burden of taking notes and generating documentation, Abridge enables clinicians to focus more fully on patients—from their very first time using the AI platform.

ABRIDGE PATIENT IMPACT METRICS

90%

OF CLINICIANS GIVE
MORE UNDIVIDED
ATTENTION TO PATIENTS

UCHICAGO MEDICINE
COREWELL HEALTH

87%

DECREASE IN PATIENT
CALL TURNAROUND TIME

REID HEALTH

77%

OF CLINICIANS
REPORTED THAT ABRIDGE
IMPROVED PATIENT CARE

KUMC

Patient Experience

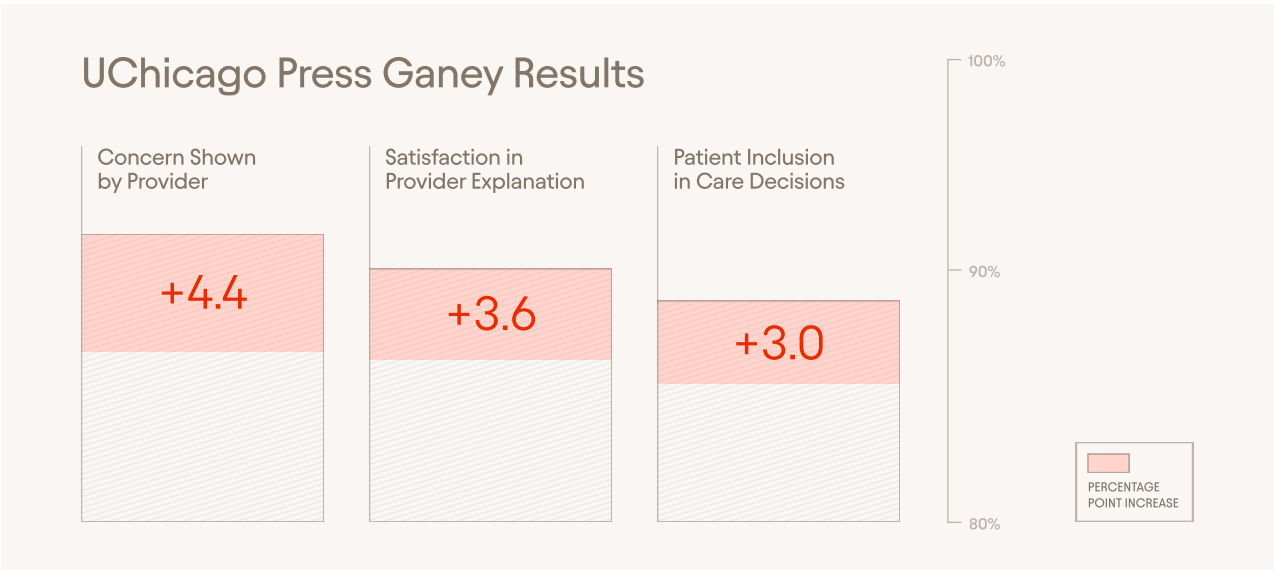
Patient experience was measured more directly at UChicago Medicine post-Abridge implementation, where **Press Ganey surveys showed astounding improvements in three crucial patient experience metrics:** concern shown by provider, satisfaction in provider explanation, and patient inclusion in care decisions.

“This shows that technology has the power to ease burdens that our clinician colleagues have been experiencing. **Half a percent increase year-over-year in these scores is generally considered an accomplishment**—in our pilot group we saw nearly an eight-fold improvement over that standard in just six weeks. Since then we've seen a sustainable 2-4 fold improvement in these scores across our general provider population when using Abridge.”

Dr. Sachin Shah
Chief Medical Information Officer, UChicago Medicine

The mechanism is reduction in cognitive load from the burden of documentation, which empowers clinicians to focus more on patients. Among many Abridge health systems that measure this effect using the NASA TLX Cognitive Load index, CHRISTUS Health saw one of the most dramatic drops at 78%.

In addition to reducing cognitive load, Abridge generates a Patient Visit Summary, which explains the contents of the visit and care plan at an eighth grade reading level. Research shows patients who are more engaged with and informed of their care have better experiences and outcomes.



Revenue Cycle Management

Abridge is transforming revenue cycle management by generating documentation that is clinically relevant and compliant at the point of care delivery. With the Contextual Reasoning Engine, Abridge addresses a common and costly problem for health systems: inaccurate and incomplete documentation, which can lead to delayed reimbursement, audit exposure, and administrative inefficiencies.

Improved documentation workflows can support measurable operational and financial outcomes across four key domains:

01 Increased wRVUs, Reported by Health Systems

02 More Accurate HCC Capture for Value-Based Care

03 Expanded Patient Access at Federally Qualified Health Centers

04 Reduced Clinician Burnout, Contributing to Higher Retention

! The Contextual Reasoning Engine enables more accurate, complete, and compliant clinical documentation by:

- Surfacing diagnoses for clinicians to review based on the contents of clinical conversations and informed by EHR data (e.g., prior note, visit diagnoses).
- Helping clinicians ensure that visit diagnoses are accurately represented and aligned with standardized coding systems, while maintaining full clinical oversight.
- Enabling clinicians to add, remove, edit, or reorder medical problems prior to note creation.
- Generating A&Ps that reflect the clinician-curated problem list, giving clinicians fine-grained control.
- Offering potential MEAT criteria related to HCC conditions already captured for clinician consideration during note creation.
- Presenting clinicians with more specific ICD-10 code options, connected directly to supporting evidence in the note via Linked Evidence—always with the clinician in control.



01

Increased wRVUs, Reported by Health Systems

By supporting more complete and specific documentation, Abridge helps enable clinician selection of appropriate E&M codes without changing the nature of clinical encounters. This also reduces administrative inefficiencies that can lead to delayed reimbursement, payer queries, or clinician frustration.

Work relative value units (wRVUs) are a standard measure of clinical effort used to assess productivity and resource utilization. In three enterprise health systems, improved documentation was associated with better reflection of clinical complexity in coding, resulting in reported wRVU increases of 11%-14%.

INCREASE IN WRVUS

↗ 11%-14%



"What [began as an initiative] focused on increasing access and reducing physician burnout has absolutely achieved those goals. But beyond that, it's also proven capable of paying for itself. When I look at some of the key metrics we track, like wRVUs, we've seen an 11% increase in the areas where we've implemented Abridge. That's a significant and measurable improvement with a defined ROI."

Matthew Cox
CFO, Corewell Health

02

More Accurate HCC Capture for Value-Based Care

Administrative inefficiencies and incomplete documentation can create challenges for value-based care organizations, particularly in capturing chronic and high-risk conditions that impact risk adjustment and reimbursement.

Abridge's platform facilitates more accurate coding, resulting in better medical record documentation and appropriate payment for care actually provided. For example, by using Abridge's platform to improve coding and documentation, [Riverside Health](#) reported a 14% increase in HCC diagnoses documented per encounter. These improvements can help more accurately reflect the complexity of care delivered, which supports appropriate reimbursement.

! What Are Hierarchical Condition Categories?

Hierarchical Condition Categories (HCCs) are a set of diagnosis groupings used by CMS and other payers to estimate a patient's expected costs and clinical complexity. Accurate HCC capture requires that the note validates the diagnosis and outlines the care provided for that diagnosis.

Abridge helps streamline the process by surfacing relevant information for chronic and high-risk conditions when generating documentation, providing cues for clinicians to review, confirm, refine, or revise the documentation as needed. This helps support clinicians in accurately capturing complexity of the care delivered.



"These changes in [more accurate HCC and E&M coding] impact our ability to accurately bill for the care we deliver, resulting in significant increases in both revenue and net margins while enabling us to offer a better patient experience."

Dr. Charles Frazier

SVP and Chief Medical Information and Innovation Officer, Riverside Health

03

Expanded Patient Access at Federally Qualified Health Centers

Abridge helps reduce cognitive load, as well as time spent on documentation, which supports clinicians' ability to focus more on delivering patient care. This is especially crucial for Federally Qualified Health Centers (FQHCs), which serve vulnerable populations with limited staffing and high demand. FQHCs differ from

other health systems in that they are mission-driven to provide care regardless of patients' ability to pay and often operate with constrained resources. Increasing access without increasing burnout is central to their sustainability.



At Yakima Valley Farm Workers Clinic, **clinicians reported an 87% increase in work satisfaction and an increase in weekly patient encounters.**

"We believe that if we weren't there that many of our patients might not get the care they need, so we're very focused on serving our patients as much as we can because the health of each of our patients is the health of humanity."

Glen Davis
Chief Operating Officer
Yakima Valley Farm Workers Clinic



At El Rio Health, **79% of clinicians reported that Abridge makes them more likely to continue practicing at El Rio for longer.**

"I am able to focus more attention on patients, while consistently completely finishing my visit note before moving on to the next patient. The decreased time needed for documentation has helped me expand access to my patients by being able to see late arrivals, walk-ins, and overbooks that I would not have otherwise been able to."

Dr. Mark Schildt
Family Medicine Physician, Chief Wellness Officer and Medical Director
El Rio Health

04

Reduced Clinician Burnout, Contributing to Higher Retention

Abridge delivers value in the form of reducing clinician burnout and turnover by decreasing documentation burden. The cost to replace a burned out physician can range between \$500,000 and \$1 million, according to the [American Medical Association](#).



55%



At both [Geisinger](#) and [Riverside Health](#), Abridge was associated with a 55% reduction in burnout among clinicians using the platform.

"We face challenges of clinician retention, efficiency, and physician burnout, and we pay a lot of attention to the latter because we need to keep our great physicians. Abridge reduces cognitive burden and pajama time while increasing access so our clinicians can actually see the patient population we serve and do what they love, which is delivering care."

Dr. Gregory Raglow
Chief Medical Informatics Officer
El Rio Health



CLINICIANS CONFIRM THE DATA:

"I love it. I'll do whatever I have to do to keep it. I've closed every single note by the end of the day, which I've never done before. It might keep me here 10 more years."

Family medicine clinician at a community health center

"It's extended my career five to ten years. There's no way I could continue this at the pace before. Now I could do this pace for 20 more years if I had to."

General practitioner at a large, regional health system

Conclusion

The Abridge ambient AI platform is rapidly transforming clinical workflows—but ambient AI is only able to drive impact when health systems achieve swift adoption and high retention among clinicians.

Abridge has developed a highly effective implementation playbook that is validated on a daily basis via increasing adoption at more than 150 health systems, across care settings, specialties, and languages.

As demonstrated through case studies and peer-reviewed research, use of Abridge is associated with several important downstream effects, including improving clinician and patient experiences while supporting revenue cycle teams. But the most important impact of Abridge for clinicians is bringing them closer to their patients and restoring their joy for delivering exceptional care. Rapid adoption and scale makes it all possible.

